Lapatinib + capecitabine

Lapatinib once daily on Days 1–21, continuously. Capecitabine twice daily on Days 1–14, then no capecitabine on Days 15–21. Repeat every 21 days while beneficial and tolerated. The capecitabine rest week is not a lapatinib rest week.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Diarrhoea / hydration

Both medicines

Report diarrhoea early, especially dizziness, poor intake or reduced urine.

2 Hands / feet

Capecitabine

Painful redness or peeling affecting activity needs prompt advice.

3 Liver / skin colour

Especially lapatinib

Report yellow skin or eyes, dark urine or severe itching.

4 Heart / chest

Both medicines

New chest pain, breathlessness, palpitations or swelling needs urgent assessment.

5 Mouth / appetite

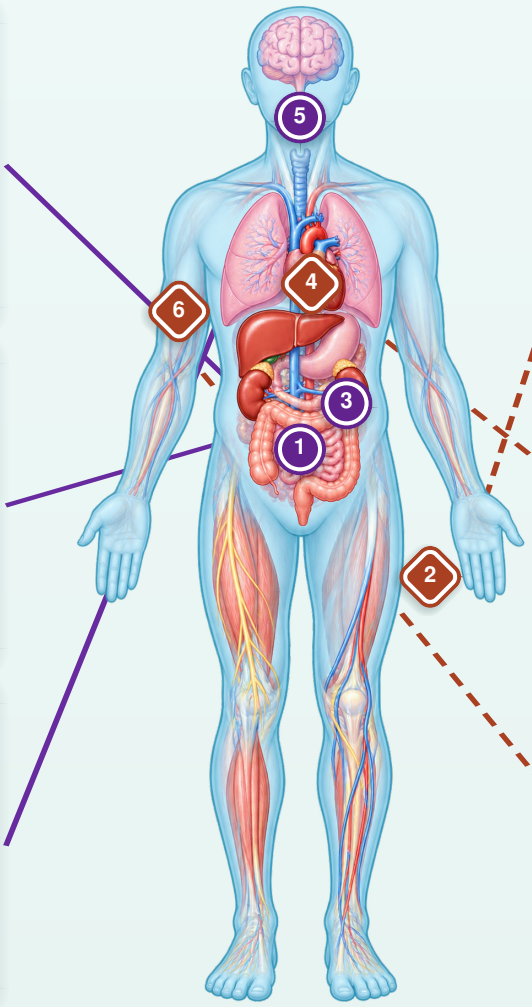
Capecitabine

Ulcers or difficulty eating and drinking need advice.

6 Rash / skin

Lapatinib

Report widespread rash, blistering or fever promptly.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

The map shows selected effects of the medicines named here. Report persistent effects from earlier treatment too.

> Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

Your treatment, at home.

Important medicine-specific effects

Lapatinib

- Diarrhoea and dehydration
- Rash
- Liver injury
- Heart pumping-function or rhythm changes
- Rare lung inflammation

Capecitabine

- Diarrhoea / dehydration
- Hand-foot syndrome
- Mouth ulcers / reduced intake
- Low blood counts and infection
- Cardiotoxicity including chest pain / ischaemia
- Severe early toxicity including dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD)-related toxicity
- Renal impairment and medicine interactions

Surgery, radiation and monitoring: open the whole-treatment map or scan the QR.

Make the plan practical

A clear tablet schedule

Take lapatinib at least one hour before or after food, consistently in the same relation to meals. Take capecitabine with food or within 30 minutes after eating. Swallow tablets whole. A pharmacist can help arrange the different food timings.

Nausea and bowel plan

These oral agents are classified as minimal-to-low emetic risk. Agree an individual symptom plan; a repeated intravenous (IV) chemotherapy antiemetic course is not automatically required.

Early symptoms matter

Call promptly for diarrhoea, dehydration, painful hands or feet, mouth ulcers or poor intake. New chest pain needs urgent assessment.

Before restarting a cycle

Review bloods, kidney and liver function, symptoms and the agreed doses. Do not restart tablets after a treatment hold without the team's instructions.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Complete reference, visual timeline and passport

https://sactcheck.com/patient/00217/

Sources and review status are available online. Follow your own prescription and hospital instructions.